

Infecções Hepatobiliares na UTI

Professor (a):

Paula S. Mesquita

Intensivista pelo HCFMUSP

Diarista da UTI da Gastroenterologia do HCFMUSP



Quais as mais comuns ?

Colangite Aguda

Obstrução da Via Biliar, geralmente relacionada a cálculo, estenose ou tumor

Abscesso piogênico

Fonte hematogênica : apendicite, diverticulite

*Ameba menos comum na prática

Colecistite Complicada

Colecistite aguda com perfuração ou necrose do órgão

Outras mais raras



Quais as mais comuns ?

Colangite Aguda

Obstrução da Via Biliar, geralmente relacionada a cálculo, estenose ou tumor

Abscesso piogênico

Fonte hematogênica : apendicite, diverticulite

*Ameba menos comum na prática

Colecistite Complicada

Colecistite aguda com perfuração ou necrose do órgão

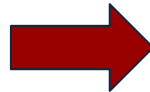
Outras mais raras



Colangite Aguda

Tríade de Charcot

Febre +
Icterícia +
Dor abdominal



Sensibilidade baixa
(50-70%)

Table 2 TG18/TG13 diagnostic criteria for acute cholangitis [4]

- A. Systemic inflammation
 - A-1. Fever and/or shaking chills
 - A-2. Laboratory data: evidence of inflammatory response
- B. Cholestasis
 - B-1. Jaundice
 - B-2. Laboratory data: abnormal liver function tests
- C. Imaging
 - C-1. Biliary dilatation
 - C-2. Evidence of the etiology on imaging (stricture, stone, stent etc.)

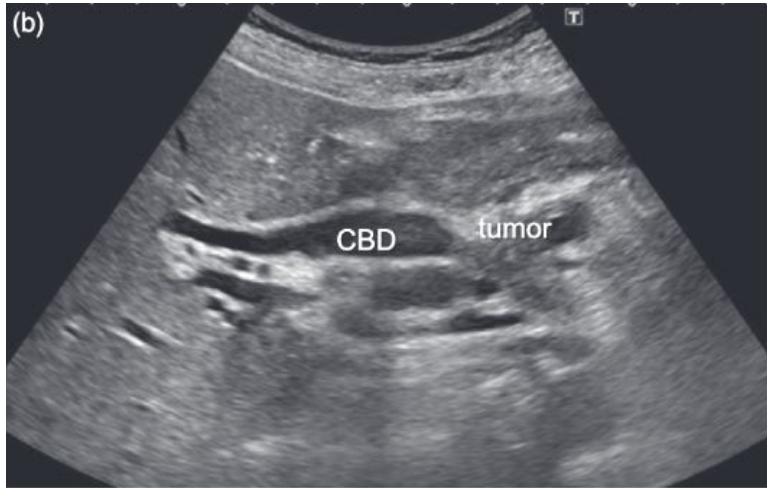
Suspected diagnosis: one item in A + one item in either B or C

Definite diagnosis: one item in A, one item in B and one item in C

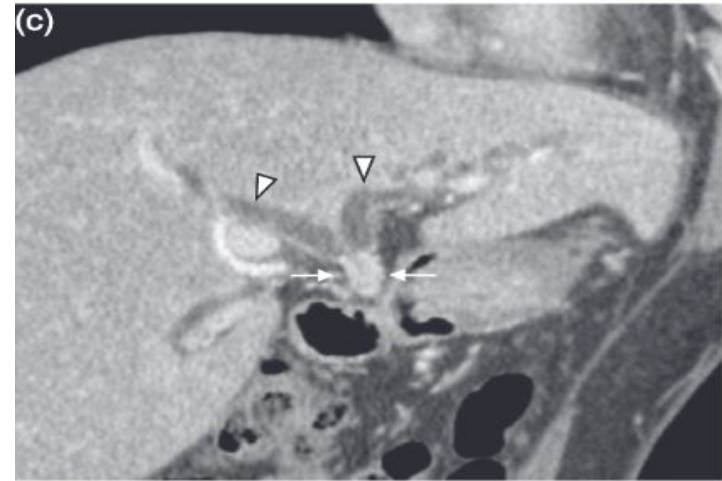
A-1	Fever		BT >38°C
A-2	Evidence of inflammatory response	WBC count ($\times 1,000/\mu\text{L}$)	<4 or >10
		CRP (mg/dL)	≥ 1
B-1	Jaundice		T-Bil ≥ 2 (mg/dL)
B-2	Abnormal liver function tests	ALP (IU)	$>1.5 \times \text{STD}^a$
		γGTP (IU)	$>1.5 \times \text{STD}^a$
		AST (IU)	$>1.5 \times \text{STD}^a$
		ALT (IU)	$>1.5 \times \text{STD}^a$



Colangite Aguda - imagens

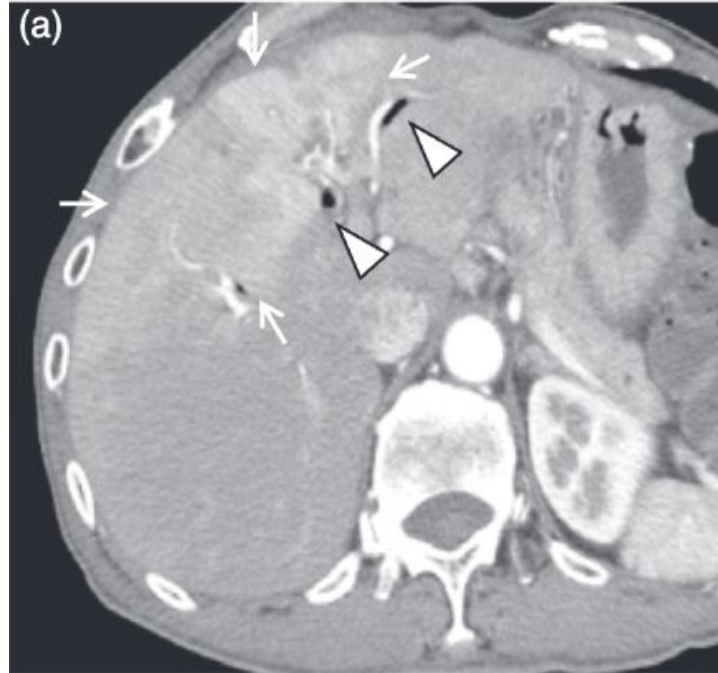


Neoplasia cabeça pâncreas

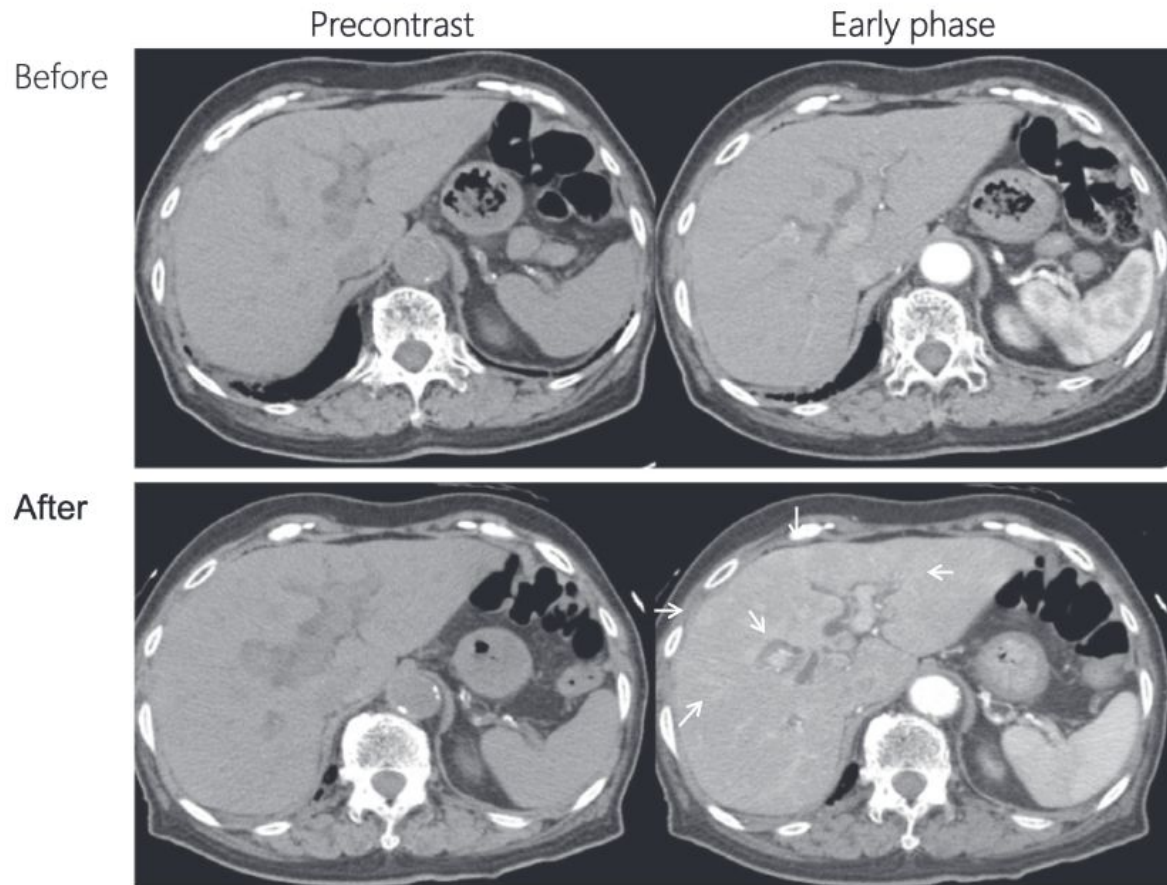


Neoplasia primária ducto biliar

Colangite Aguda - imagens



Colangite + abscesso



Colangite Aguda - Gravidade

Table 4 TG18/TG13 severity assessment criteria for acute cholangitis [4]

Grade III (severe) acute cholangitis

“Grade III” acute cholangitis is defined as acute cholangitis that is associated with the onset of dysfunction at least in any one of the following organs/systems:

1. Cardiovascular dysfunction: hypotension requiring dopamine $\geq 5 \mu\text{g/kg}$ per min, or any dose of norepinephrine
2. Neurological dysfunction: disturbance of consciousness
3. Respiratory dysfunction: $\text{PaO}_2/\text{FiO}_2$ ratio < 300
4. Renal dysfunction: oliguria, serum creatinine $> 2.0 \text{ mg/dl}$
5. Hepatic dysfunction: PT-INR > 1.5
6. Hematological dysfunction: platelet count $< 100,000/\text{mm}^3$

Grade II (moderate) acute cholangitis

“Grade II” acute cholangitis is associated with any two of the following conditions:

1. Abnormal WBC count ($> 12,000/\text{mm}^3$, $< 4,000/\text{mm}^3$)
2. High fever ($\geq 39^\circ\text{C}$)
3. Age (≥ 75 years old)
4. Hyperbilirubinemia (total bilirubin $\geq 5 \text{ mg/dl}$)
5. Hypoalbuminemia ($< \text{STD}^a \times 0.7$)

Grade I (mild) acute cholangitis

“Grade I” acute cholangitis does not meet the criteria of “Grade III (severe)” or “Grade II (moderate)” acute cholangitis at initial diagnosis.



Colangite Aguda - Tratamento

Drenagem biliar



Antibióticos



Colangite Aguda - Tratamento

Drenagem biliar



Antibióticos

Urgente ?



Urgente ?

Table 3 Thirty-day mortality rate relevant to the timing of biliary drainage and severity grading by TG13 [17]

Severity grade	30-day mortality according to the timing or absence of biliary drainage					
	Urgent biliary drainage			Urgent or early biliary drainage		
	Within 24 h (n = 2,709)	After 24 h or absence (n = 3,354)	P-value	Within 48 h (n = 3,730)	After 48 h or absence (n = 2,333)	P-value
Grade III (n = 1,521)	5.4% (42/781)	4.9% (36/740)	0.727	4.9% (50/1,017)	5.6% (28/504)	0.622
Grade II (n = 2,019)	1.7% (16/939)	3.4% (37/1,080)	<0.05	2.0% (25/1,272)	3.7% (28/747)	<0.05
Grade I (n = 2,523)	1.3% (13/989)	1.2% (18/1,534)	0.853	1.1% (16/1,441)	1.4% (15/1,082)	0.586
Total (n = 6,063)	2.6% (71/2,709)	2.7% (91/3,354)	0.873	2.4% (91/3,730)	3.0% (71/2,333)	0.164

Urgent performed on the admission day (within 24 h), *early* performed on the day following admission (24–48 h)

J HepatobiliaryPancreat Sci. 2017;24:329–37



Colangite Aguda - Tratamento

Drenagem biliar



Antibióticos

Urgente ?

Se o paciente está séptico
o controle de foco sempre
é prioridade !



Colangite Aguda - Tratamento

Drenagem biliar



Antibióticos

- CPRE
- Drenagem transparieto-hepática percutânea
- Cirúrgica



Colangite Aguda - Tratamento

COLETAR MATERIAL PARA CULTURA !

Drenagem biliar



Antibióticos

- CPRE
- Drenagem transparieto-hepática percutânea
- Cirúrgica



Colangite Aguda - Tratamento

Drenagem biliar



Antibióticos

Cobertura para gram negativo e anaeróbios

Ex.: Ceftriaxone e Metronidazol



Colangite Aguda - Tratamento

Drenagem biliar



Antibióticos

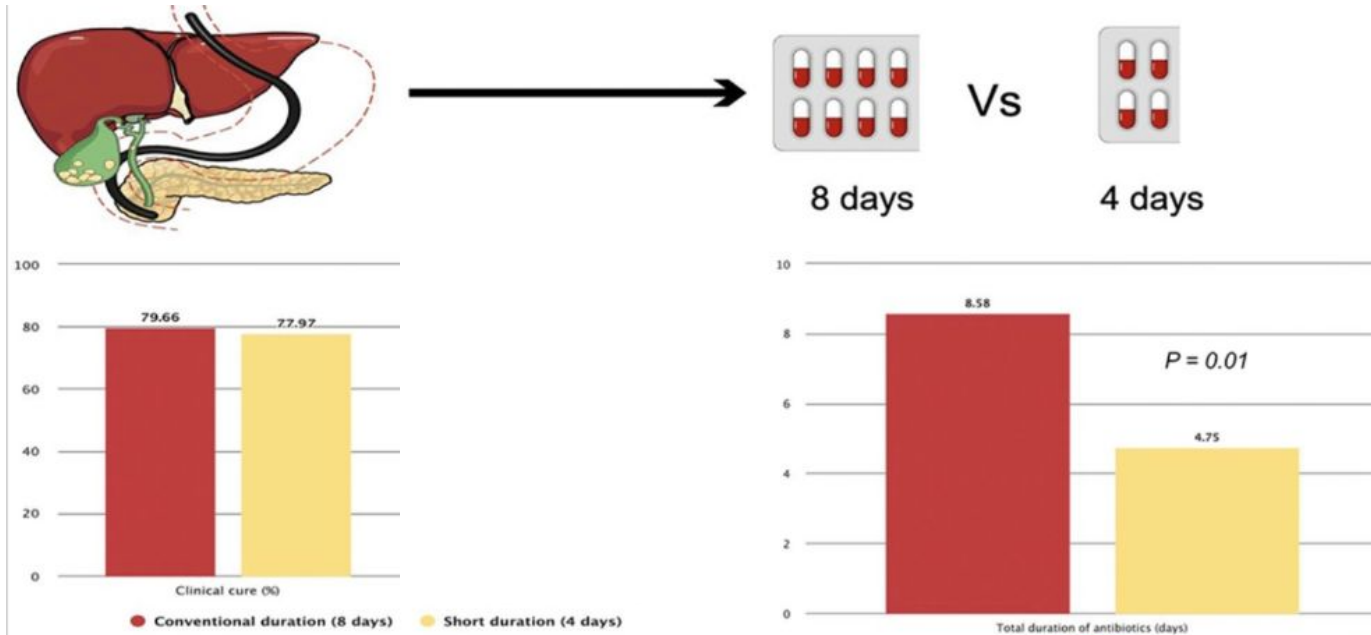
Cobertura para gram negativo e anaeróbios

Ex.: Ceftriaxone e
Metronidazol

Tempo ?



Conventional vs Short Duration of Antibiotics in Patients With Moderate or Severe Cholangitis: Noninferiority Randomized Trial



Deshidi S et al. *Am J Gastroenterol*. 2024. doi:10.14309/ajg.0000000000002499

AJG The American Journal of
GASTROENTEROLOGY



Colangite Aguda - Tratamento

Drenagem biliar



Antibióticos

Once the source of infection is controlled, antimicrobial therapy for patients with acute cholangitis is recommended for the duration of 4 to 7 days. (Recommendation 1, level C)



Colangite Aguda - Tratamento

Drenagem biliar



Antibióticos

Once the source of infection is controlled, antimicrobial therapy for patients with acute cholangitis is recommended for the duration of 4 to 7 days. (Recommendation 1, level C)

Antimicrobial therapy for patients with Grade I and II acute cholecystitis is recommended only before and at the time of surgery. (Recommendation 1, level B)

Once the source of infection is controlled, antimicrobial therapy for patients with Grade III acute cholecystitis is recommended for the duration of 4 to 7 days. (Recommendation 2, level D)



Colecistite Aguda

Grau	Definição	Conduta
I (Leve)	- Inflamação limitada à vesícula	Colecistectomia precoce
II (Moderada)	≥ 1 dos critérios: - Leucócitos > 18.000 - Massa palpável em QSD - Sintomas > 72h - Inflamação local marcada	Colecistectomia precoce é seguro
III (Grave)	Disfunção orgânica em ≥ 1 sistema: - Cardiovascular: vasopressor - Neurológico: alteração consciência - Respiratório: $PaO_2/FiO_2 < 300$ - Renal: $Cr > 2 \text{ mg/dL}$ - Hepático: $INR > 1,5$ - Hematológico: Plaquetas < 100.000	Suporte intensivo + Colecistostomia



Colecistite Aguda - Drenagem Percutânea



Fonte própria



Abscesso Hepático

Piogênico

Relacionado à obstrução biliar,
tromboflebites (diverticulite,
apendicite com acometimento
vascular)

Amebiano

Áreas endêmicas;
Pouco comum na UTI.

UTI

Choque séptico de foco abdominal



Abscesso Hepático

Piogênico

Table 1. Causes of pyogenic liver abscess

Biliary tract	Iatrogenic injury of liver/biliary tract
Cholangitis	Liver biopsy
Choledocholithiasis	Blocked biliary stent
Cholangiocarcinoma	Radiofrequency ablation/hemoembolization in the presence of infected bile
Ischemic cholangitis	Pancreatoduodenectomy
Congenital biliary diseases	Liver transplantation
Portal vein	Hepatic artery
Appendicitis	Dental infection
Diverticulitis	Bacterial endocarditis
Crohn's disease	Pyelonephritis
	Osteitis
Direct extension of:	Secondary infection of liver
Gall bladder empyema	Liver cyst
Perforated peptic ulcer	Biliary cyst
Subphrenic abscess	Metastases
Kidney abscess	
Liver trauma	Cryptogenic



Abscesso Hepático

UTI



Fonte própria



Abscesso Hepático - Tratamento

Drenagem



Antibióticos

Percutânea na grande maioria

Cirurgia :

> 5 cm, múltiplos, ruptura com peritonite

Ceftriaxone + Metronidazol

2-4 sem



Hidatidose

Equinococose forma cística.

- Ingestão de água e alimentos contaminados.



Obrigada

Dúvidas :

gastrointensiva@gmail.com

